

ENCOUNTER 5 OF 5

Date of Service: 2025-03-15 | Type: OUTPATIENT VISIT | Specialty: Neurology | Setting: Outpatient | Acuity: LOW

Attending Provider:	Dr. Marcus Webb, MD	Location:	New Amandaville Neurology Associates
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Chief Complaint

Follow-up evaluation approximately 12 weeks status-post left tarsal tunnel decompression, for reassessment of residual sensory symptoms.

Interval History

Ms. Gardner returns to neurology clinic approximately 12 weeks following surgical excision of a ganglion cyst with decompression of the left posterior tibial nerve. She reports substantial ongoing improvement in her symptoms since the last visit. The pins-and-needles sensation in her left toes has almost completely resolved, and she now rates any residual intermittent tingling at 1/10, occurring only rarely after prolonged periods of standing at work, which is a marked improvement from her pre-operative baseline of constant 3/10 symptoms. She denies any residual numbness that interferes with her daily activities, denies weakness, and denies falls. She has completed her post-operative physical therapy course and has returned to her full pre-operative work duties, including prolonged standing, without significant limitation.

She denies any new neurologic complaints, denies symptoms in the contralateral extremity, and denies bowel or bladder dysfunction. She continues to take her home medications for hypertension, asthma, and hypothyroidism without any recent changes or adverse effects.

Review of Systems (12-point)

System	Findings
Constitutional	Denies fever, chills, or unintentional weight loss.
HEENT	History of seasonal allergies; denies vision changes or hearing loss.
Cardiovascular	History of hypertension, well controlled; denies chest pain or palpitations.
Respiratory	History of mild intermittent asthma; denies current dyspnea or wheeze.
Gastrointestinal	Reports occasional constipation; denies nausea, vomiting, or GI bleeding.
Genitourinary	Reports occasional urinary frequency; denies dysuria or hematuria.
Musculoskeletal	Reports mild left knee stiffness; denies acute joint swelling elsewhere.
Neurological	Reports numbness/tingling in left toes; denies headache, seizure, or syncope.
Psychiatric	History of generalized anxiety, stable on no pharmacotherapy; denies SI/HI.
Endocrine	History of hypothyroidism, on levothyroxine; denies heat/cold intolerance.
Integumentary	Reports occasional dry skin; denies rash or new lesions.
Hematologic/Lymphatic	History of mild anemia; denies easy bruising or lymphadenopathy.

Past Medical History

Chronic conditions include essential hypertension (dx. 2016), mild intermittent asthma (dx. childhood), and primary hypothyroidism (dx. 2019, on levothyroxine replacement). Surgical history is notable for a low-transverse cesarean section (2010) and a tonsillectomy (childhood). The patient was hospitalized previously for community-acquired

pneumonia (2015) and following a motor vehicle collision (2018) with no long-term sequelae reported at that time.

Family History

Mother: hypertension, type 2 diabetes mellitus, living. Father: coronary artery disease, myocardial infarction at age 61, living. One sibling (sister): hypothyroidism. No known family history of malignancy, bleeding disorders, or hereditary neuropathy.

Social History

The patient is a non-smoker with no history of tobacco use and denies use of illicit substances. She reports occasional social alcohol use (1-2 drinks per week) and regular caffeine intake (1-2 cups of coffee daily). She works as an administrative coordinator, a role that requires prolonged standing and walking throughout the day. She is married and lives with her spouse and two children. She reports a generally sedentary exercise pattern with occasional weekend walking.

Current Medications

Medication	Dose	Route	Frequency	Indication
Aspirin	81 mg	PO	Daily	Cardioprotective
Lisinopril	10 mg	PO	Daily	Hypertension
Albuterol HFA	2.5 mg (nebulized equiv.)	Inhaled	PRN	Asthma
Levothyroxine	50 mcg	PO	Daily	Hypothyroidism
Acetaminophen	1000 mg	PO	PRN	Pain

Allergies

No known drug allergies (NKDA). No known food or environmental allergies of clinical significance documented.

Vital Signs

Parameter	Value	Parameter	Value
Blood Pressure	120/70 mmHg	Heart Rate	64 bpm
Temperature	98.1 F	Respiratory Rate	15 breaths/min
Pain Score	0-1/10	SpO2	99% RA
Weight	152 lbs	Height	5 ft 5 in
BMI	25.3 kg/m2		

Physical Examination

General

Well-appearing female in no acute distress, ambulating normally without assistive devices.

Wound/Incision

The medial ankle incision has healed well with a thin, mature, non-tender scar. No residual swelling, erythema, or induration.

Neurological Examination

Cranial nerves II-XII grossly intact. Motor strength 5/5 throughout bilateral lower extremities including tibialis anterior, gastrocnemius, and extensor hallucis longus. Sensation to light touch and pinprick now grossly intact and symmetric throughout the left foot and toes, a marked improvement compared to prior exams, with only trace subjective diminished sensation immediately adjacent to the surgical scar reported by the patient but not clearly reproducible on formal testing today. Tinel's sign at the tarsal tunnel is now negative. Deep tendon reflexes 2+ and symmetric. Gait is normal-based, non-antalgic; heel walk and toe walk performed without difficulty. Romberg negative.

Repeat Diagnostic Studies

Test	Result	Reference Range	Flag
Repeat Nerve Conduction Study, left posterior tibial nerve	Near-normalized conduction velocity	Normal conduction velocity	Improved / Near Normal

Repeat nerve conduction study of the left posterior tibial nerve demonstrates near-normalized conduction velocity across the tarsal tunnel, consistent with the patient's reported clinical improvement and confirming successful decompression.

Assessment

1. Resolved/resolving left tarsal tunnel neuropathy, status post surgical decompression, with excellent clinical and electrodiagnostic improvement. No evidence of recurrence.
2. Hypertension, controlled.
3. Mild intermittent asthma, controlled.
4. Primary hypothyroidism, controlled.

Plan

1. No further physical therapy required at this time; encourage continued independent home stretching program as needed.
2. No further neurology follow-up scheduled at this time; the patient may be discharged from active neurology follow-up.
3. Continue routine primary care follow-up for management of hypertension, asthma, and hypothyroidism.
4. Return to neurology clinic as needed if symptoms recur or new neurologic symptoms develop.
5. Continue home medications without change.

Patient Education / Instructions

The patient was informed of the excellent clinical and electrodiagnostic recovery and the low likelihood of recurrence. She was advised to contact the clinic should any numbness, tingling, or weakness recur or worsen in the future. She expressed satisfaction with her surgical outcome and had no further questions at today's visit. This concludes the active treatment course for this condition.

Care Team

Name	Role	Contact
Marcus Webb, MD	Attending Neurologist	555-201-7710
Priya Chandrasekaran, RN	Neurology Clinic Nurse	555-201-7715
Dr. Anita Reyes, MD	Orthopedic Surgery (co-managing, visit concluded)	555-201-4410
Dr. Susan Whitfield, MD	Primary Care Physician	555-201-3300

Pain & Functional Assessment Questionnaire

Question	Response
Average residual symptom severity over past week (0-10)	1/10
Frequency of residual tingling	Rare, only after prolonged standing
Interference with work duties	None
Interference with sleep	None
Return to baseline activity level	Yes, fully returned
Overall satisfaction with treatment outcome (0-10)	9/10

Symptom Trend Summary Across Encounters

Time	BP	HR	RR	Temp	SpO2	Pain
2024-06-15	128/72	68	16	98.2F	97% RA	3/10
2024-08-10	124/70	70	16	97.9F	98% RA	3/10
2024-12-21	125/66	63	16	98.0F	96% RA	3/10
2024-12-28	122/68	66	16	97.8F	98% RA	2/10
2025-03-15	120/70	64	15	98.1F	99% RA	0-1/10

The table above summarizes vital signs and reported pain/symptom severity across all five encounters in this chart, demonstrating a stable cardiopulmonary status throughout the treatment course and a clear downward trend in reported symptom severity from a baseline of 3/10 constant symptoms to near-complete resolution (0-1/10, intermittent) following surgical decompression and post-operative recovery.

Long-Term Follow-Up Recommendations

Given the excellent surgical and electrodiagnostic outcome, no routine long-term neurology surveillance is recommended for this condition. The patient was counseled that a small minority of tarsal tunnel decompression patients may experience recurrence of symptoms years later, most commonly due to scar tissue formation or, rarely, cyst recurrence, and that such recurrence would warrant repeat clinical evaluation and, if indicated, repeat electrodiagnostic testing and imaging. She was also encouraged to maintain supportive footwear during prolonged standing at work as a general preventive measure, and to continue the independent home stretching and nerve-gliding exercises she learned during physical therapy on an as-needed basis.

Care Coordination Note / Chart Closure

This visit concludes the active neurology and orthopedic surgery treatment course for the patient's left tarsal tunnel neuropathy. A summary letter was sent to the patient's primary care physician, Dr. Susan Whitfield, documenting the full course of treatment from initial presentation through surgical resolution, for inclusion in the primary care longitudinal record. The patient was advised to continue routine primary care follow-up for her chronic conditions (hypertension, asthma, hypothyroidism) and was provided standard instructions to re-contact either specialty clinic directly should any related symptoms recur in the future. No further specialty appointments are scheduled at this time.

Electronically signed by: **Marcus Webb, MD**

Department: Neurology

Signed: 2025-03-15 3:30 PM